

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Office Visit - Mar 27, 2026

with Anand Bery, MD at Mass Eye and Ear Otoneurology Service



Notes from Care Team



[Progress Notes by Anand Bery, MD at 3/27/2026 1:00 PM](#)

OTO-NEUROLOGY / VESTIBULAR NEUROLOGY CONSULTATION

Daniel S May is a 36 y.o. who comes for neuro-vestibular consultation regarding dizziness.

Dear Dr. Krause, Gregory Evan, MD ,

HISTORY

History of Present Illness:

There is no strong history of motion sensitivity.

He had two migraines in the summer 2025. Moderate headache, impacted function, preferred to go somewhere dark and quiet.

Longstanding ear and sinus issues

He has been doing budesonide and sinus rinses. He finds he gets room spinning x 1 minute when he bends forward to perform the nasal rinse

A month ago, he first began to experience dizziness, accompanied with nausea/vomiting, chills, fever

Currently

Subtle dizziness at present, always occurs with blowing his nose.

Triggers

Blowing his nose

Busy visual environments do not seem to be triggers.

Valsalva otherwise does not seem to be a consistent trigger.

There has been no positional vertigo.

Has not noticed significant change in hearing recently.

Medications Tried for Vestibular/Balance Issues:

as above

Please see his dizzy questionnaire, which has been scanned into Epic, for further details on review of systems/past medical history/social history/family history.

Review of systems:

Systems were reviewed. Significant findings described above in HPI.

Past Medical History:

has a past medical history of Trauma to eye.

Family History:

family history is not on file.

Social History:

See chart and patient questionnaire.

Medications:

No outpatient medications have been marked as taking for the 3/27/26 encounter (Appointment) with Bery, Anand Kumar, MD.

Allergies:

has no known allergies.

EXAMINATION

Appearance, affect, and higher cognitive function

He was well-appearing and well-groomed with normal affect. He was oriented with good recent and remote memory and gave a coherent history with intact concentration. There was no aphasia, apraxia, or agnosia and there was a good fund of knowledge.

Gait and balance

Casual gait was normal. Standing with feet together was possible with eyes open and closed. Tandem stance was possible with eyes open and only briefly eyes closed.

Coordination

Finger to nose testing was normal.

Screening vision

Visual fields were deferred.

Bedside ocular motor and vestibular exam

Range of eye movements: normal.

Ocular alignment: normal.

Convergence: normal .

Straight ahead fixation: normal.

Eccentric fixation: normal.

Rebound nystagmus: absent

Saccades: promptly initiated, accurate and of normal speed.

Smooth pursuit: normal

Vestibulo-ocular reflex suppression (VORS): normal.

Optokinetic responses: normal.

Visually-enhanced vestibulo-ocular reflex: normal.

Head impulse test (HIT): normal in the planes of the horizontal canals.

Anisocoria in both light and dark with R pupil larger

Normal facial strength and sensation

VIDEO-OCULOGRAPHY (VOG) and video Head Impulse Test (vHIT) **(3/27/2026)**

See separate report.

STUDIES REVIEWED

I personally reviewed all relevant imaging and diagnostics, with relevant findings below:

Auditory and outside vestibular testing

Audiogram: see below

Patient: DANIEL MAY (PMRN:10105654130)

Get Report

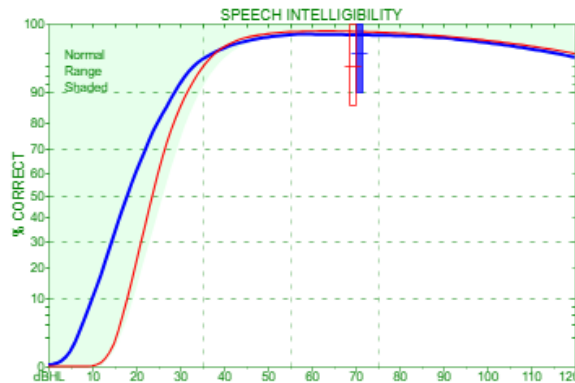
☐ Left ☐ Right ☒ Air

DOB: 10/11/1989 Age: 35 y.o. DOS: 10/4/2025

☒ Bone ☒ SF

Audiologist: Anna M. Keenan Au.D., CCC-A Refer: KUSH PANARA, M.D.

Assist: Clinic: Audiology

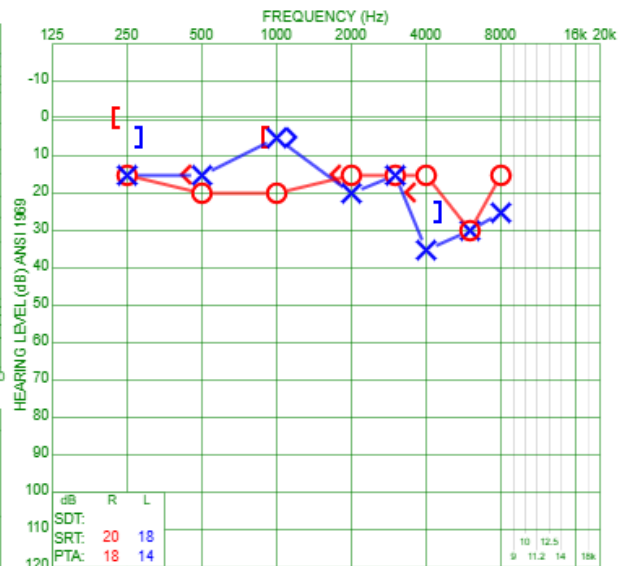


SPEECH TESTING			
RIGHT		LEFT	
LIST	dBHL Score	LIST	dBHL Score
W22 3A	70 96%	W22 4A	70 98%

SYMBOLS	RIGHT	LEFT
Speech Intelligibility Index Prediction		
Speech Score 95%CD		
Air Conduction		
AC Masked		
Bone Conduction		
BC Masked		
Sound Field		
Response at Limit		
No Response		
Acoustic Reflex Ipsilateral Probe		
Acoustic Reflex Contralateral Probe		

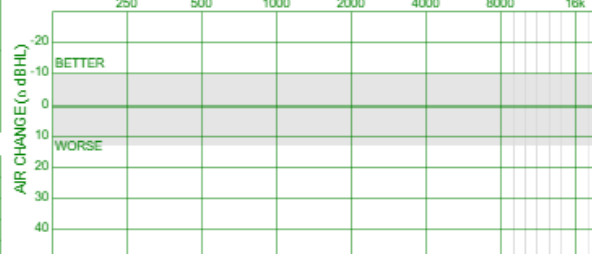
TYMPANOMETRY		
FEATURE	RIGHT	LEFT
VOLUME	1.0 ml	0.7 ml
PRESSURE	-5 daPa	-181 daPa
MOBILITY	2.2 ml	0.6 ml
SHAPE	Normal	Normal

REFLEX DECAY		
FREQUENCY	RIGHT	LEFT
500 Hz		
1000 Hz		
2000 Hz		
4000 Hz		



LowF-PTA (dB): 18 12

HighF-PTA (dB): 19 26



REASON FOR TEST

Eustachian tube dysfunction

HISTORY

Patient is seen for suspected eustachian tube dysfunction. He reports aural fullness and popping bilaterally, left ear worse. Patient has a history of sinus surgeries. He denies additional otologic complaints.

HEARING MEASUREMENT

TDH and insert earphones were used for testing. Ipsilateral acoustic reflexes to broadband stimulation are absent in the right ear.

IMPRESSIONS

Tympanometry results are consistent with normal middle ear pressure and hypermobility in the right ear, and negative middle ear pressure with normal mobility in the left ear. Conductive overlay is noted at 250 and 1000 Hz in the right ear. Absence of acoustic reflex suggests the conductive components are of middle ear origin. The patient may experience communication difficulties in adverse listening situations, and is expected to benefit from the use of good communication strategies.

CONSULTATION

Results were reviewed with the patient.

Imaging

MRI brain IAC ordered

Labs

No results found for: "NA", "K", "CL", "CO2", "BUN", "CREATININE", "GLU", "ALKPHOS", "PROT", "ALBUMIN", "WBC", "HCT", "PLT", "HGBA1C", "TSH", "FREET4", "CHOL", "TRIG", "HDL", "LDLCAL"

No results found for: "HOMOCYSTEINE", "MMA", "FOLATE", "VITB1", "VITE", "VITA", "VITD25OH"

ASSESSMENT AND PLAN

Recurrent, short episodes of dizziness/vertigo (lasting seconds-mins) that seem to be reliably triggered by blowing his nose and/or nasal sinus rinses. It is not triggered by sound or other kinds of Valsalva.

On exam and testing:

- No peripheral vestibular loss
- Mild left-beating nystagmus in left DH, without symptoms, not suggestive of BPPV
- No central eye movement findings

Additionally

There was no clear Valsalva (pinched nose or closed glottis)-induced dizziness, making a third window phenomenon less likely

There was no nystagmus nor symptoms in bow (replicating the position in which he does nasal sinus rinses

Plan

MRI brain IAC views

If sound/pressure remain consistent triggers, consider CT temporal bone

Issues discussed: diagnosis and implications on future health, education on risk factors given his condition, and test results and/or further testing

I personally spent a total of 60 minutes on care for this patient on the date of the encounter. This includes face-to-face time during the visit as well as non face-to-face time spent on chart review, documentation and care coordination.

Follow-up: As needed

Sincerely,

Anand Bery, MD
Ocular Motor and Vestibular Oto-Neurologist
Mass Eye and Ear / Harvard Medical School

No orders of the defined types were placed in this encounter.

Cc List:

Mr. May

No address on file

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